DRUGS · VERTIGO AND MÉNIÈRE'S DISEASE

Vestibular sedatives and antiemetics

Vestibular sedatives suppress vertigo and nausea in an acute attack: prochlorperazine (a phenothiazine dopamine antagonist), cinnarizine (an antihistamine with calcium-channel effects), cyclizine and promethazine (antihistamines). They give symptomatic relief in vestibular neuritis, Ménière's attacks and vestibular migraine, but they slow central compensation, so they should be used for days only. In the elderly they cause falls, confusion and drug-induced parkinsonism.

1 CHAPTER ONE At a glance

The drugs on one page.

1.1 Summary

VESTIBULAR SEDATIVES AND ANTIEMETICS

	SUMMARY
Agents	Prochlorperazine; cinnarizine; cyclizine; promethazine; betahistine is not a vestibular sedative
How they work	Prochlorperazine blocks D2 receptors in the chemoreceptor trigger zone; antihistamines block H1 and muscarinic receptors in the vestibular nuclei and vomiting centre
ENT uses	Acute vertigo with nausea (vestibular neuritis, Ménière's attack, vestibular migraine, post-operative ear surgery); motion sickness (cinnarizine, promethazine, hyoscine)
Adult dose	Prochlorperazine 5 to 10 mg orally up to three times daily, 3 to 6 mg buccally twice daily, or 12.5 mg intramuscularly; cinnarizine 30 mg three times daily; cyclizine 50 mg up to three times daily orally or intravenously; promethazine 25 mg at night or up to three times daily
Child dose	Prochlorperazine is not for children under 1 year and is used with care in older children; cinnarizine 5 to 12 years 15 mg three times daily; cyclizine and promethazine by age

	SUMMARY
Duration	Ideally no more than 3 days
Avoid	Prochlorperazine in Parkinson's disease, and in the elderly where possible; all in glaucoma and urinary retention (anticholinergic)

2 CHAPTER TWO Using them in ENT

Indications.

2.1 Acute vestibular syndrome

In acute vestibular neuritis, short-term prochlorperazine (buccal if vomiting) for the first days, then stop and start **early mobilisation and vestibular rehabilitation**; continued suppression delays compensation and prolongs symptoms. Always check for a **central cause** first (HINTS examination, new neurological signs, inability to walk).

2.2 Ménière's attacks

Buccal prochlorperazine or oral cinnarizine during an attack; they do not prevent attacks.

3 CHAPTER THREE Safety

What goes wrong.

3.1 Adverse effects

- **Extrapyramidal effects** with prochlorperazine: acute dystonia (oculogyric crisis, especially in young women), parkinsonism, akathisia; tardive dyskinesia with long-term use; neuroleptic malignant syndrome rarely.

- Sedation, dry mouth, blurred vision, urinary retention and constipation; confusion and falls in the elderly.
- Cinnarizine with long-term use can cause parkinsonism and depression in the elderly.

3.2 Interactions

Other sedatives and alcohol; QT-prolonging drugs (promethazine, prochlorperazine).

4 CHAPTER FOUR For the examination

What the examiner wants to hear.

4.1 Viva essentials

WHAT THE EXAMINER IS LOOKING FOR

- Why vestibular sedatives should be used for **days, not weeks** (central compensation).
- Extrapyramidal side effects and their treatment (procyclidine for dystonia).
- Why to examine for a central cause before treating vertigo symptomatically.

4.2 Common pitfalls

COMMON PITFALLS

- Repeat prescriptions of prochlorperazine for chronic dizziness in an elderly patient.
- Missing a cerebellar stroke presenting as acute vertigo.

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